Royal Devon University Healthcare NHS Foundation Trust

LocSSIP Guideline for: Non-medical Practitioner Spinal Injections at The Nightingale Hospital (NGE)

SUMMARY

This guideline outlines the process to be followed for Spinal injections carried out by Non-medical Practitioners at the NGE, an invasive procedure linked to the principles of safe site surgery.

The guideline has been written as a result of National Safety Standards for Invasive Procedures (NatSSIPs), national guidance which requires detailed management plans for all invasive procedures undertaken in an outpatient, treatment room or ward setting.

PUBLICATION DETAILS

Author of LocSSIP	Faye McCluskey – ACP
Division/ Department responsible for Clinical Guideline	Spinal Orthopaedics (PEOC)
In consultation with	Spinal Orthopaedic Colleagues and Nightingale Hospital Colleagues.
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Contact for Review:	Faye McClusky Extended Scope Practitioner faye.mccluskey1@nhs.net

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1 INTRODUCTION

1.1 The Royal Devon University Healthcare Trust is committed to providing high quality patient centred care and a safe environment for all patients undergoing any invasive procedure.

2. BACKGROUND

2.1 An NHS England Patient Safety Alert (September 2015) recognises that 'surgical' Never Events are relevant to all clinical settings where invasive procedures are undertaken and requires Trusts to broaden the scope of evidence-based standards that build on the WHO Surgical Safety Checklist approach in order to address many of the underlying causes of Never Events.

2.2 This guideline is intended to build on the principles outlined in the Trust's Safe Surgery and Interventional Procedures Policy (Eastern Services) and ensure a standardised approach to the Spinal Injection service carried out by Non-Medical Practitioners in a non-theatre setting, i.e. outpatients, SWAOC Fluoroscopy suite, treatment / clinic rooms etc.

3. DEFINITIONS

3.1 Nerve root injections – Local anaesthetic and steroid injected next to one or more affected spinal nerves. Contrast dye is used to ascertain correct positioning.

3.2 Caudal Epidurals – Injection of a volume of local anaesthetic, steroid and saline into the epidural space via the sacral hiatus.

3.3 MUA and coccygeal injections – Manipulation under anaesthetic followed by an injection of local anaesthetic and steroid to the sacrococcygeal junction.

3.4 ACP / Trainee ACP – Advanced Clinical Practitioner / Trainee Advanced Clinical Practitioner

4. GOVERNANCE AND AUDIT

4.1 The Spinal Orthopaedic Services specialty governance/audit groups will monitor the implementation of this guideline and commission an annual audit of adherence (see Appendix 4). Cycles of improvement will be undertaken on a continuous basis and monitored by the governance group to ensure meaningful, relevant development of this LocSSIP in line with the national guidance (NatSSIP).

5. DOCUMENTATION OF INVASIVE PROCEDURES

5.1 Safety checklists have been developed for use when invasive procedures are undertaken in Outpatient settings – see Appendix, 1.

5.2 For OPD procedures these should be recorded in the medical notes on EPIC and the clinic letter and the checklist stored in the medical notes on EPIC.

6. Workforce, Duties and Responsibilities, Training

6.1 Minimum staffing requirements for these invasive procedures are:

- 1 x ACP or Trainee ACP
- 2 x Nurses
- 1 x Radiographer

Additional requirements and responsibilities:

- All staff allocated to the list must be up to date with their anaphylaxis training.
- Medical oversight is provided by the Orthopaedic specialty team. Out of hours, this will be via the on-call orthopaedic team, accessible through switchboard.
- During in-hours sessions, if two theatre lists are running, the third-on anaesthetist may be available to support in the event of a medical emergency.
- For GLANSO lists and out-of-hours procedures, medical advice is provided by the base specialty or the on-call team (Orthopaedics via switchboard).
- Immediate support during the list is provided by the team present.
    - At least one team member must hold current ILS certification, and all team members must be in date with anaphylaxis training.

6.2 It is the responsibility of each member of staff involved in the procedures outlined to:

- to comply with the standards set out in this guidance
- to work within their own competence
- to escalate any concerns regarding own and other's competence
- to report all incidents of non-compliance (including near miss events) using the Trust's Incident Reporting procedures on Datix (Incident Reporting, Analysing, Investigating and Learning Policy). Incidents will be discussed at the Spinal Orthopaedics Audit/ Governance Group and any identified actions resulting from incidents implemented. Any incident that occurred at the Nightingale site will also be discussed at the Nightingale governance group and be included in external reporting to the ICB.

6.3 Training

All members of staff involved in the delivery of invasive procedures outlined in this guideline must complete either (1) the Trust's training as per Section 22 of the overarching Trust Safe Surgery and Interventional Procedures Policy or (2) the LocSSIP training presentation and verify their agreement to work and communicate as part of a team and to speak out if they have any concerns about patient safety.

7. SCHEDULING

7.1 These procedures will be performed in a timely fashion during Outpatient clinics in SWAOC after the patient has been appropriately prepared. Optimum (safe) numbers of patients will be 12 in an all-day session starting at 9.00. Each slot will be 30 minutes (45 for bilateral injections). There will be a break for lunch at 13:00 with the list finishing at 16:00 to allow time for debriefing, checks and cleaning.

8. HANDOVERS AND INFORMATION TRANSFER

8.1 **Pre-procedure** – see Checklists in Appendix 1.
Prior to the procedure, the information as outlined on the Checklists will be checked with the patient to ensure that identity is confirmed, the patient understands the nature of the procedure, Risks (Appendix 2), laterality, and that verbal consent is obtained

8.2 **Post-Procedure**
Relevant information will be handed over to the staff (clinic or ward nurse) responsible for discharging the patient from clinic/ward who will ensure that the patient receives the appropriate written/verbal information. The information will include laterality of the injection, any complications during the procedure and advice regarding any additional needs they may have prior to discharge.

9. PROCEDURAL VERIFICATION AND SITE MARKING

9.1 The practitioner will ensure that the correct site and side (if applicable) for the procedure is agreed at the combined Sign In / Time Out safety check and that it corresponds to the request form or clinic letters (where applicable).

10. SAFETY BRIEFING

10.1 The team involved in the invasive procedure will have introduced themselves.

10.2 The purpose of the brief is to discuss the procedure. Where there are back to back listed patients where there are no team changes it is acceptable to discuss the whole of the sessions planned list.

10.3 The area used should be quiet, allow confidentiality and be free from interruptions.

10.4 It is accepted that operationally the pre-brief would discuss the whole proposed planned list.

10.5 The brief may be led by any confident designated member of the team.

10.6 All members of the team are named for the session and their roles are identified.

10.7 Any anticipated milestones or challenges must be considered, and plans put in place if necessary.

10.8 Equipment and pharmaceutical treatment stocks and checks should have already been performed and any issues must be highlighted, and relevant actions must be taken (See Appendix 1).

11. SIGN IN AND TIME OUT – COMBINED SAFETY CHECK

11.1 The team involved in the invasive procedure will participate in the combined safety check as per the Checklists ensuring that the following information is confirmed and agreed by the team with the involvement of the patient: (See Appendix 1)

- Correct patient identity (all in-patients to have an identity wristband that correctly identifies them to their care (NPSA 2009) as per the Identification of Patients Policy
- Appropriate verbal consent in place (as per the Consent to Treatment and Examination Policy
- Correct site (identification see Section 9) and procedure to be undertaken agreed.
- That patients meet the agreed safety criteria (Appendix 3)
- This is to be followed by 'Stop before you block' Protocol in line with Safe Surgery Policy (appendix 9).

12. POST PROCEDURE CHECKS

12.1 The team involved in the invasive procedure will participate in the Post-procedure checks as per Sign out. The procedure is correctly recorded, and post-procedure management plan discussed and relevant information given to the patient. (See Appendix 1)

13. DEBRIEFING

13.1 The debrief may be led by any confident team member.

13.2 The staff member who has performed the procedure must be present.

13.3 All members of the team should have noted any key points for consideration at the debrief that arose throughout the session. These should be raised and presented for discussion at the debrief.

13.4 All members present at debrief must feel comfortable enough to contribute to the debrief discussion and raise any concerns or questions.

13.5 Any questions included at the debrief are professional and not personal and are designed to facilitate reflection, shared learning and make improvements where appropriate.

13.6 The leader of the debrief must also cover "what went well".

13.7 Examples of questions for discussion and consideration at debrief.

- Communication – any issues for improvement?
- Team harmony – for example, was the best use of skills utilised?
- Planning – were there any planning issues, i.e. missing stock, how can we improve?
- Equipment failure – any issues?
- Have any problems been resolved?
- Will anything need to be addressed prior to the next clinic / treatment session?
- Who is taking the responsibility to ensure actions?
- Identification of training or development that could be of benefit may also be considered.

14. MONITORING COMPLIANCE WITH THIS GUIDELINE

14.1 See audit tools in Appendix 4. In the OPD, a 12 monthly case-note review of minimum of 10 patient procedures to ensure all elements of the checklists are being completed (2 observed and 8 retrospectively from documentation and radiology imaging).

14.2 The Consultant Mentor, will present the audit results for review and discussion at the relevant specialty governance group meetings on an annual basis in the Autumn each year, or by exception in the event of any patient safety or risk issues being raised by the team from the above audit activity.

15. ASSOCIATED CLINICAL GUIDELINES OR POLICIES/PROCEDURES

- Safe Surgery and Interventional Procedures Policy
- Consent to Treatment and Examination Policy
- Identification of Patients Policy
- Incident Reporting, Analysing, Investigating and Learning Policy and Procedures

LocSSIP Agreement

• I have read the relevant local guideline (LocSSIP) and the revised Safe Surgery and Interventional Procedures Policy and understand my role within it. • I agree to work and communicate as part of a team and to speak out if I have any concerns about patient safety.		
Name of LocSSIP: Non-medical Practitioner Spinal Injections at SWAOC at The Nightingale Hospital (NGE)		
Name:	**Signature:**	**Designation:**
Department:	**Division**:	

16 PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE STANDARD OPERATING PROCEDURE/ GUIDELINE

16.1 To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
Annual case-note review of minimum of 10 patient procedures to ensure all elements of the checklists are being completed (2 observed and 8 retrospectively from documentation and radiology imaging).	See audit tool	Spinal Governance
Incident reporting in relation to the procedure	Datix review	Spinal Governance

Appendix 1 – Safety Checklist (Stop before you block)

Safety Briefing:

- Have all team members introduced themselves by name and role?
- Are there any changes to the published operating list?
- How are specimens taken to be managed?
- Is the theatre adequately staffed and are any changes expected?
- Please confirm any equipment or implants that will be required.
- Are the anaesthetic machines checked and ready for use?
- Are there any patient allergies?
- Are there any patient infection control issues?

Sign in:

- Patient ID to be confirmed by asking the patient (using positive identification methods) and checking all ID bands in situ against the notes**.**
- Ask the patient what procedure they are expecting.
- Check that verbal (or written) consent has been obtained.
- Check site and site of procedure (if relevant).
- Ask patient about allergies and record them.
- Check for infection control alerts and specify what they are on the checklist.
- Confirm that all necessary equipment is ready and sterile.

Time out:

- Time Out must occur as close to the invasive procedure as practicably possible

- Time Out must be performed by two staff members (minimum)
- All other activity must stop while you 'pause' to conduct a final verbal verification with the patient. This can be completed at the same time as Sign In.

Sign Out:

- Positive patient identification
- Procedure carried out and recorded
- Drugs administered during procedure
- Has the patient been given the appropriate information?
- Are all swabs, sharps and instruments accounted for?
- Specimen form completed if applicable

Stop Before You Block process:

Stop Before You Block is a three-step process of undertaking any peripheral nerve block or local anaesthetic infiltration: **Step 1 = Prep; Step 2 = Stop; Step 3 = Block Step 1: 'Prep'**;

- **Step 1: Preparation Sign-in;**
  - On the arrival of the patient in the Anaesthetic Room, Block Room or Operating Theatre, the WHO 'Sign In' is performed, including the confirmation of the patient identity, the operative consent form and that the operative site has been marked. General anaesthesia may or may not be administered first (or at all): regardless the following steps should be taken in relation to the conduct of the block.
  - **Drugs and Equipment;** the Blocker draws up the local anaesthetic solution and places the labelled syringe, along with a suitable nerve block needle, in a dedicated tray/container, separate from intravenous drugs. This tray/container is then handed over to the Assistant, out of the Blocker's immediate reach.
  - **Positioning:** The Blocker and Assistant should position the patient and equipment (e.g., ultrasound machine, peripheral nerve stimulator) in the final position ready for the block. Where necessary for the conduct of the block, any dressing or plasters casts should be removed.
  - A 'pre-scan' ultrasound might be undertaken at this stage. At this point, or before, the Blocker may perform a preliminary 'scoping' ultrasound scan (if relevant). Preparing site/gloving: The Blocker dons' gloves and prepares the site (cleaning, draping) suitable for the block.
  - Where drapes cover the surgical site mark, the Assistant should ensure they can later reveal the mark for the Stop moment. Ideally, the surgical site mark should always be visible.

- **Step 2: Stop moment** The stop moment is a two-person step that happens only after preparation is complete and thus immediately before needle insertion.
  - When the Blocker is ready (e.g., with the nerve(s) to be blocked located by ultrasound), the Blocker formally announces they have completed all preparation and are ready to block using a Safe Surgery and Interventional Procedures Policy Ratified by: Safety and Risk Committee: 14-07-21 Review date: June 2026 Page 48 of 50 consistent form of words: ***"I've completed my prep; let's Stop Before You Block".***
  - The Assistant similarly should reply***: "OK, let's Stop Before You Block".***

- The Blocker and Blocker's Assistant together should then check the block side by viewing the surgical site mark, verbally confirming the correct side: the Assistant reconciles this with the consent form. If the patient is awake and un-sedated, they may also confirm the side is correct.

- **Step 3: Block** Only when the correct side is confirmed does the Assistant hand the tray/container to the Blocker. The Blocker immediately performs the block. Any delay between handing back the tray and/or performing the block should require the Blocker and Assistant to re-start the SBYB process at Step 1.
    - Delays might arise due to movement of the patient/site to be blocked; patient instability and need to resuscitate, or interruptions from visitors to the room or by telephone calls. This re-start is to re-create a situation in which the block immediately follows the handing of tray from Assistant to Blocker.
    - Re-start might be at the very start of the Preparation stage (e.g., if the site has become unsterile) or later, depending on circumstance; but will always involve first handing the tray back to the Assistant.
    - Recording and Analysing the process After the block, the Blocker and Assistant should record that the SBYB protocol was followed. The process allows analysis of compliance with the SOP for audit purposes, and also permits analysis of a wrong side block by serving as a reference against which any adverse event can later be mapped.

Appendix 2 – Spinal Injections Risks

Risks of Spinal Injections: checklist prior to administering an injection

Potential Risks to Injection
Allergy to iodine
Allergy to Local Anaesthetics
Warfarin or other anti-coagulant therapy
Clotting/bleeding disorders
Current infection- local or systemic
Diabetes
Pregnancy
Allergy to Shellfish
Anxiety/Phobia

Potential Complications from Spinal Injection

Potential Complication Checklist
Anaphylaxis
Vaso-vagal event
Dural puncture
Bleed (epidural haematoma)

Neural impairment/injury
Altered glycaemic control in diabetics
Infection
Altered menstruation
Facial red-ness
Fat atrophy
Post injection pain
Injection site skin bleaching
Local anaesthetic toxicity
Complete spinal anaesthetic
Cauda equina syndrome

These risks and potential complications are discussed within the Consent form on EPIC.

Appendix 3 – Safety Guideline and Patient Criteria

Safety Guidelines and Patient Criteria

The injection will NOT be performed if the patient presents with the following:

Systolic Blood Pressure >190mmHg or <80 mmHg
Diastolic Blood pressure <100mmHg
Diabetes - Capillary Blood sugar >12mmol
Warfarin - INR >1.5
High temperature - Pyrexia >37.5 degrees
Continued anticoagulant/antiplatelet use
Within 3 weeks of stopping antibiotics, or high suspicion of infection
Pregnant

Precaution:
BMI > 40

Anticoagulant Management for the ESP Lumbar Injection List

Low Risk Patients:

Previous DVT/PE over 12/12 ago, stable AF (as per anti coagulation policy)

Anticoagulant	Last Dose
Rivaroxaban Dabigatran (Prodaxa) Apixaban (Eliquis) Enoxaparin Fondaparinux Edoxaban	5 days
Clopidogrel (Plavix) (Ticlodipine, Prasugre, ,Dipyridamole, Ticagrelor)	10 days
Warfarin	5 days INR <1.5
Aspirin < 75 mg	10 days
Aspirin > 75 mg	10 days

High Risk Patients:

Mechanical Heart Valve or recent Stroke/TIA (6/12) Seek Specialist Advise

Compiled from RD&E Anticoagulation Clinical Guideline 2019
Approved by Spinal Governance Group 06/09/1

Appendix 4 – Audit Tool

Audit tool - 2 observed injections and 8 injections audited from documentation and radiology imaging annually.

<u>Audit tool for Non-medical Spinal Injectors - Observed</u>

Date:			**Comments / action points**
Team Briefing	Was Team brief carried out? Were appropriate questions asked? Were appropriate team members present?	**Y/N** **Y/N** **Y/N**	
Sign in	Sign in completed as per trust protocol?	**Y/N**	
Time out	Time out completed as per trust protocol?	**Y/N**	

Sign out	Sign out completed as per trust protocol?	**Y/N**	
Debrief	Debrief completed as per trust protocol?	**Y/N**	
Spinal injection Observed	NRB, CE or MUA coccyx Sterile technique observed? Correct injection site observed?	 **Y/N** **Y/N**	
Documentation	Correct documentation carried out?	**Y/N**	

Audit tool for Non-medical Spinal Injectors - Documentation and imaging audit

Date:		**Comments / action points**
Patient discussed at VC prior to injection?	**Y/N**	
Pre-procedure /Consent clearly documented?	**Y/N**	
Post op notes completed?	**Y/N**	
Fluoroscopy imaging available?	**Y/N**	
Correct site injected?	**Y/N**	
Follow up completed by referring practitioner?	**Y/N**	
Name and signature of auditor		

Faye McCluskey Spinal ESP 16/08/24

Appendix 5 - Communication Plan

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the guideline/SOP	All SWAOC staff and Diagnostic staff involved with the procedure, Anaesthetic staff working at the Nightingale
The key changes if a revised document	New Document
The key objectives	This guideline is intended to build on the principles outlined in the Trust's Safe Surgery and Interventional Procedures Policy (Eastern Services) and ensure a standardised approach to

	the Spinal Injection service carried out by Non-Medical Practitioners in a non-theatre setting, i.e. outpatients, SWAOC Fluoroscopy suite, treatment / clinic rooms etc
How new staff will be made aware of the procedure/guideline and manager action	Cascade by email from manager, induction process
Specific Issues to be raised with staff	Safety Check lists and stop before you block
Training available to staff	Support to be obtained from the line manager for targeted training
Any other requirements	
Issues following Equality Impact Assessment (if any)	NA
Location of hard / electronic copy of the document etc.	On the Trust intranet The shared drive for the Nightingale.